

1. Administrative & Study Identification

Official Study Title: A Prospective, Multicenter, Single-arm, Open-label, Phase IV, Post-authorization Interventional Study to Assess the Safety and Efficacy of Asciminib in Indian Patients With Ph+ CML-CP (Without T315I Mutation), Previously Treated With Two or More Tyrosine Kinase Inhibitors and Ph+ CML-CP With T315I Mutation
Brief Study Title: Clinical Study of Asciminib in Previously Treated Indian Patients With Ph+ CML-CP Without T315I Mutation and in Patients With Ph+ CML-CP With T315I Mutation
Short Title/Acronym: ASC4INDIA **Protocol Number:** CABL001A2005 **NCT Number:** NCT06427811 **Posting ID:** 186518564905229663 **Sponsor/Company Name:** Novartis Pharmaceuticals / Novartis **Investigational Product:** Asciminib (ABL001) 40 mg film-coated tablets (Trade Name: Scemblix, ATC Code: L01EA06) **Phase of Development:** Phase 4 **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Medical Monitor:** Novartis Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To confirm the safety and efficacy of asciminib in Indian patients with Philadelphia chromosome-positive chronic myeloid leukemia in chronic phase (Ph+ CML-CP) (without T315I mutation) who have been previously treated with 2 or more tyrosine kinase inhibitors (TKIs), and in patients with Ph+ CML-CP with the T315I mutation. **Secondary Objectives:** To evaluate early molecular response (EMR), complete hematologic response (CHR), dose interruptions/reductions/discontinuations, Major Molecular Response (MMR) rate, Deep molecular responses (MR4, MR4.5), time to response, and duration of response.

2.2 Background & Rationale The Drugs Controller General of India (DCGI) granted approval for Asciminib film-coated 40 mg tablets on October 20, 2023, with the condition to perform a Phase IV clinical study in Indian patients. This post-authorization study is conducted to confirm the clinical benefit, safety, and tolerability of asciminib in the specified target population within India per the local Prescribing Information (PI).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Post-authorization) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** Non-Randomized

3.2 Planned Enrollment & Duration Target Enrollment:
Approximately 85 evaluable patients **Number of Sites:** Multicenter (India) **Estimated Study Start:** 15-Oct-2024 **Estimated Primary Completion:** 23-Apr-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female participants ≥ 18 years of age with a confirmed diagnosis of Ph+ CML-CP. 2. Evidence of typical BCR::ABL1 transcript (e14a2 and/or e13a2) amenable to standardized RQ-PCR quantification. 3. ECOG performance status of 0, 1, or 2. 4. **Without T315I mutation:** Previous treatment with at least 2 prior ATP-site TKIs (e.g., imatinib, nilotinib, bosutinib, dasatinib, ponatinib) experiencing failure or intolerance. **With T315I mutation:** Documented T315I mutation at any time point, irrespective of previous lines of therapy.

4.2 Exclusion Criteria 1. Known second chronic phase of CML after previous progression to CML-Accelerated Phase (AP) or Blast Phase (BC). 2. Previous treatment with a hematopoietic stem-cell transplantation. 3. Cardiac or cardiac repolarization abnormality, including history of myocardial infarction (MI), angina pectoris, or CABG within 6 months prior to starting study treatment. 4. Severe and/or uncontrolled concurrent medical disease that could pose unacceptable safety risks or compromise protocol compliance.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * *Participants without T315I mutation:* 80 mg once daily at approximately the same time each day OR 40 mg twice daily per Investigator's discretion at approximately 12-hour intervals. * *Participants with T315I mutation:* 200 mg twice daily at approximately 12-hour intervals. **Route of Administration:** Oral (film-coated tablets) **Duration of Treatment:** Up to 6 months of treatment.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures for managing CML-associated symptoms that are not contraindicated. **Prohibited:** Other concurrent anti-cancer therapies, investigational treatments, strong CYP3A inhibitors/inducers, or medications with a known risk for Torsades de Pointes or significant QTcF prolongation.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Up to Day -21 to 0 | Informed Consent, Medical History, T315I Mutation testing (Sanger/PCR/

NGS), ECG, ECOG, BCR::ABL1 PCR | | **Baseline** | Day 0 | Treatment Allocation, First Dose Dispensation, Baseline Safety Labs | | **Interim** | Months 1 to 6 | Safety Lab monitoring, Hematologic response assessment, Efficacy Assessments (PCR), AE Evaluation | | **End of Study** | 30 days post-last dose | Final Safety Follow-up, AE resolution tracking, End of Study declarations |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. Frequency and severity of adverse events (AEs) / serious AEs (SAEs) in participants with Ph+ CML-CP (without T315I mutation), previously treated with 2 or more TKIs up to 6 months. 2. Frequency and severity of AEs/SAEs in participants with Ph+ CML-CP with T315I mutation up to 6 months.

7.2 Secondary & Exploratory Endpoints * Percentage of participants with dose interruptions, reductions, and discontinuation (without T315I mutation). * Percentage of participants achieving a complete hematologic response (CHR) at 3 and 6 months (without T315I mutation). * Percentage of participants achieving early molecular response (EMR) at 3 and 6 months (without T315I mutation). * Major Molecular Response (MMR) rate achieved by the end of the treatment period. * Deep molecular response rates (MR4 and MR4.5). * Time to response and Duration of response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Method of collection involves continuous monitoring for hematologic toxicity (absolute neutrophil count, platelets), cardiovascular events (QTcF prolongation), fatigue, and nausea via scheduled laboratory assessments and patient interviews. **Serious Adverse Events (SAEs):** Serious and severe events (Grade 3 or 4 toxicity) must be reported within standard 24-hour timelines. Recurrent hematologic toxicities may mandate dose interruptions or reductions as per the India PI. **Data Monitoring Committee (DMC):** Internal/External safety review to monitor TKI-associated toxicities and cardiovascular safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy measures (MMR); Safety Set for patients receiving at least one dose of asciminib for AE/SAE evaluation. **Sample Size Justification:** The cohort of 85 targeted enrollments is statistically modeled to descriptively evaluate post-marketing safety and confirm efficacy patterns in Indian patients aligned with the regulatory stipulations. **Handling of Missing Data:**

Standard approaches such as assigning non-responder status for missing PCR data points at primary endpoint evaluations.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), the Declaration of Helsinki, and is mandated by the Drugs Controller General of India (DCGI).

Monitoring Plan: Risk-based monitoring plan comprising periodic onsite clinical trial monitoring and remote data verification.

Audit Trail: All patient information, mutation status, and molecular responses will be captured in validated eCRF systems with comprehensive audit trails.

11. Study Identifiers & Governance

Posting ID: 186518564905229663 **Primary ID (Protocol):**

CABL001A2005 **Registry Number:** NCT06427811 **Sponsor/Lead**

Organization: Novartis Pharmaceuticals **Study Title:** ASC4INDIA

Official Regulatory Title: A Prospective, Multicenter, Single-arm, Open-label, Phase IV, Post-authorization Interventional Study to Assess the Safety and Efficacy of Asciminib in Indian Patients With Ph+ CML-CP (Without T315I Mutation), Previously Treated With Two or More Tyrosine Kinase Inhibitors and Ph+ CML-CP With T315I Mutation **Trial Status:** ACTIVE_NOT_RECRUITING

12. Clinical Framework (CML Specific)

Disease Area: Philadelphia Chromosome-Positive Chronic Myeloid Leukemia in Chronic Phase **Preferred UMLS Name:** Chronic myeloid leukemia, Ph-positive, chronic phase **MeSH Code:** D015942 **SNOMED ID:** 109968000 **Diagnosis Threshold:** Confirmed Ph+ CML-CP via RQ-PCR; confirmed T315I mutation status **Medical Methodology:** Specifically Targeting the ABL Myristoyl Pocket (STAMP) BCR-ABL1 Inhibitor Therapy **Optimization Target:** Achievement of Major Molecular Response (MMR) and Deep Molecular Response (MR4, MR4.5)

13. Study Procedures & Methodology

Management Pathway: Standardized Ph+ CML management tailored by T315I mutation presence (dosing adjustment) **Education Protocol (HCP):** Training on Phase IV reporting obligations, protocol-specific dose modification algorithms, and cardiovascular safety monitoring. **Education Protocol (Patient):** Counseling on twice-daily/once-daily adherence regimens and continuous medication logs. **Quality Audit Mechanism:** Centralized evaluation of BCR::ABL1 transcript levels and ongoing safety signal detection.

14. Observation & Follow-Up Schedule

Total Duration: 6-month treatment period + 30-day safety follow-up
Onsite Visit Cadence: Baseline, Months 1, 2, 3, 4, 5, 6, and Safety Follow-up
Remote Monitoring Frequency: Supplemental tele-visits as defined by study site operational plans.
Checklist Review Interval: Laboratory and concomitant medication review prior to every cycle's drug dispensation.

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					